A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Barry Teperman
The Medical Clinic

Dear Dr Teperman,

Re: Ms Anabel Shears
DOB: 15/06/1950 Phone: 0418 328 157
Address: 4 Willis Street, North Balwyn, Vic, 3104
Email: avtshearscarter@bigpond.com

Summary

- **Resolution of significant abdominal pain and improvement in bowel habit.**
- **Review of pancreatic imaging at St Vincent's Hospital confirming no evidence of malignancy or active autoimmune pancreatitis.**
- **Continuation of nortriptyline and esomeprazole.**
- **For review in July following rheumatology assessment.**
- **No action required by you.**

I had the pleasure of conducting a review with Anabel Shears. I am pleased to report that the debilitating abdominal pain she was experiencing earlier this year has largely resolved. She noted that the pain disappeared around mid-April, and her bowel habit has also significantly improved. Previously, she was experiencing up to five evacuations per morning; she is now having approximately two formed movements with better volume and improved control.

We discussed her current medications. Anabel is continuing with nortriptyline, which I suspect has been instrumental in both managing her abdominal pain and regulating her bowel transit by slowing contractility and reducing visceral sensitivity. She has also successfully reduced her esomeprazole to 20 mg at night. She continues to use kiwi fruit to assist with regularity. Regarding side effects, she is experiencing some dry mouth and chronic dry eyes. We also discussed her balance; while she has noted some instability, this appears related to her history of sudden deafness and tinnitus rather than a new neurological issue, though she remains mindful of the risk of falls.

While her gastrointestinal symptoms improved as her prednisolone was tapered, she is now experiencing a recurrence of proximal muscle pain and weakness in her legs, hips, and upper arms. She is due for further blood tests and a review with her rheumatologist in mid-June to reassess a possible diagnosis of polymyalgia rheumatica.

Finally, I had her imaging reviewed at St Vincent's. The consensus remains that while her initial CT scan was suggestive of autoimmune pancreatitis, the subsequent MRI was normal and did not show any evidence of malignancy or inflammatory changes. Given her excellent clinical response and the lack of suspicious findings on MRI, we will continue to defer any invasive investigations such as an endoscopic ultrasound or pancreatic

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biopsy.

Impression and Plan The current strategy is to maintain her clinical stability. We agreed to continue the nortriptyline for the time being, as we do not wish to change too many variables while her rheumatological management is being optimised. Once her musculoskeletal symptoms are stabilised and her medication regimen is fixed, we can consider a slow taper of the nortriptyline, which I usually recommend continuing for nine to twelve months in the context of chronic pain. I have planned to review Anabel again in July, after her next appointment withher rheumatologist

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Mr James Keck, Ms Angela Khera

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065